

Body:	World Health Organization
Issue concerning:	Strengthening access to the health system for displaced and conflict-affected populations
Main submitter:	People's Republic of China
Co submitters:	The Kingdom of Saudi Arabia, Republic of Sudan, The Republic of Kenya.
Signatories:	Republic of Yemen, Federal Republic of Nigeria, Syrian Arab Republic, Ukraine, Romania, Canada, Arab Republic of Egypt, The Commonwealth of Australia, The United Kingdom of Great Britain and Northern Island, Japan, Republic of Türkiye, The French Republic, Federal Republic of Germany, The Republic of South Africa, Libya.

The World Health Organization,

Affirming the commitment of Member States to promote the highest standard of health for all people,

Keeping in mind the need for international cooperation,

Further recalling healthcare is a fundamental human right, especially for vulnerable groups like refugees and ethnic minorities affected by war,

Alarmed by the lack of resources, medical professionals, and new technologies in LEDCs.

Aware of the importance of primary health care (PHC) towards reaching universal health care (UHC),

Bearing in mind resolution 46/182 created by the General Assembly in 1991, establishing the Inter-Agency Standing Committee (IASC), the highest-level humanitarian coordination forum of the UN, which gathers leaders from 19 UN agencies in order to prioritize actions and gathers resources for crisis,

Acknowledging the fact that a considerable part of the global population currently lacks access to basic health care assistance, the urgent issue regarding limited to zero access

healthcare causing serious lack of medical support, Recognizing the current existence of 117 million refugees, asylum seekers and internally displaced persons,

Aware of Goal 3 on “Good Health and Well-being” of the internationally accepted fundamental agreement Sustainable Development Goals of the United Nation,

Taking into account the urgent need to address these inequalities,

Expecting all Member States to recognize access to healthcare as a fundamental human right, ensuring inclusive policies that prioritize refugees and vulnerable populations affected by conflict,

1. **Seeks** accountability and transparency in the utilization of humanitarian aid by implementing and supervising regulations in every country with the guidance of the United Nations Office for the Coordination of Humanitarian Affairs (OCHA) such as:
 - a. using technology to track in real time the number of supplies present in each institution public or private,
 - b. promote appropriate levels of transparency through context-sensitive mechanisms, such as periodic reporting, independent audits, or oversight bodies, while respecting national sovereignty, data protection standards, and security considerations,
 - c. encourage the timely and accessible communication of relevant governmental policies and actions concerning the health system, particularly those affecting emergency and conflict-related medical services, are made publicly available in a timely and accessible manner;
2. **Encourages** the partner countries to support and invest in programmes such as the Horizon Europe Innovative Health Initiative (IHI) to support and invest in Non-Governmental Organizations like United Nations Children's Fund (UNICEF), Internal Rescue Committee (IRC), International Medical Corps (IMC), and Partners In Health (PIH) that offer direct help to unprivileged areas such as:
 - a. delivering medical aid,
 - b. providing emergency and trauma care services,
 - c. calling for remote clinics to be placed in conflict-affected areas therefore allowing basic health care services for those who cannot reach hospitals,
 - d. the training and capacity-building of healthcare workers, local volunteers, and hospital staff to provide patient care, clinical support, and first aid,

- e. innovation challenges that support healthcare improvements and innovations;
3. **Proposes** to commit to further safeguarding the humanitarian corridors already existent in compliance with international law, in order to assure rapid and uninterrupted, unconditioned aid, free from matters of:
 - a. political interests,
 - b. cultural and religious biases,
 - c. geographical position;
 4. **Encourages** situational adaptations for the methods of aid transport for the specific environments it is requested in, in order to provide healthcare as efficiently as possible and to strengthen access to it, such as:
 - a. specialised logistics and Infrastructure (e.g. Multimodal Transport, Pre-positioning Supplies), by providing a bigger number of ambulances, equipped with advanced medical tools,
 - b. operational and security measure (e.g. Air Charters, Localizing Logistics),
 - c. encouraging the appropriate use of new technologies, such as the drone-delivery of medical supplies, in respect to the local context and depending on the infrastructure capacity of each state,
 - d. technology and data solutions (e.g. Digital Cash Transfer);
 5. **Calls for** the strengthening of national healthcare systems in conflict-affected regions through international funding and infrastructure development by:
 - a. encouraging the adaptation of aid transport for the specific environments it is requested in, as afore-mentioned in OC4, in order to provide healthcare as efficiently as possible and to strengthen access,
 - b. encouraging member countries to support and invest in NGOs like UNICEF and the Red Cross,
 - c. supports the implementation of multilingual healthcare services, including translation support and culturally adapted care, to reduce communication barriers for displaced populations;
 6. **Proposes** deeper connections with non-governmental organizations (NGOs) in order to provide healthcare services to those displaced by conflicts through funding collaborations, capacity building activities, and coordinating efforts that can accommodate a larger amount of people for the insurance of culturally relevant healthcare interventions;

7. **Proposes** the creation of international laws to protect the emergency health teams that only come to aid those in critical conditions and cannot reach hospitals in time following the guidelines of the International Committee of the Red Cross (ICRC) regarding respecting and protecting healthcare in armed conflicts and in situations not covered by International Humanitarian Law, these teams being neutral and can lead to:
 - a. finance health infrastructure in rural or isolated areas who can't secure proper health standards for its people,
 - b. building properly hospitals and clinics,
 - c. training the local staff in order to assure proper qualifications for giving first aid;
8. **Recommends** the further improvement of coordinated global frameworks, such as the World Health Organization, to ensure equitable distribution of resources and sustainable healthcare solutions for displaced people;
 - a. hold coordination meetings regularly to update each other and decide priorities together:
 - i. these meetings should happen monthly, or even weekly in case of a medical crisis,
 - ii. the attendants would be representatives from national health ministries or local authorities,
 - iii. representatives share updates on current situations, like number of patients, urgent health issues and available resources,
 - b. create health coordination hubs in crisis regions, as following:
 - i. in these centers, organizations like WHO, national governments and NGOs work together,
 - ii. this would ensure immediate help and constant security for the populations living there, being protected and looked over at all times;
9. **Calls for** the implementation of a secure, encrypted database for refugees medical records, documented in the form of Digital Health Passports made especially for IDPs in all Member States in order to protect the fundamental right to health by:
 - a. developing a digital platform, through which asylum seekers and refugees can be continuously informed by medical updates,
 - b. creating national databases that share real time information, editable by the medical healthcare workers that are committed to create personal medical profile for each patient with a private 10 digit security number based given at the creation of it,

- c. implementing the blockchain technology for a decentralized database, accessible to all sanitary institutions around the world;
10. **Encourages** the member countries to work towards reaching the 2030 Immunization Agenda to reach complete herd immunity and stopping the spread of infectious diseases by supporting:
- a. lessons from the educational systems about the importance and effectiveness of modern vaccines, along with how they function,
 - b. governmental transparency with the population about vaccines,
 - c. community-led awareness campaigns with local trusted messengers, such as local religious leaders and teachers, to combat vaccine hesitancy and misinformation and to educate communities on them;
11. **Proposes** task shifting, where tasks are redistributed from doctors to nurses, pharmacists and community health workers so that healthcare personnel can be used more efficiently by:
- a. suggesting the use of experienced healthcare workers that are not currently in charge, offering them flexible jobs,
 - b. recommending implementing a community volunteer program to enhance health care efficiency through formalized first aid education,
 - c. assigning each head of the hospital to responsibly distribute the roles,
 - d. encouraging countries to achieve vaccine self-sufficiency and reducing reliance on external support,
 - e. calls for strengthened international cooperation to support healthcare infrastructure, expand workforce capacity and improve access to essential medical supplies particularly in under served regions;
12. **Proposes** the implementation of a Community Based Health Insurance (CBHI) funded through a combination of voluntary contributions, international grants and state subsidies specifically made for displaced people, guaranteeing access to essential medicine and surgeries with the UN also as a partial funder;
13. **Supports** the use of digital health technologies to:
- a. reach populations in inaccessible areas,
 - b. assure fast emergency response, to create efficient healthcare and overcome language barriers;
14. **Calls for** the expansion of mental health and psychosocial support services as to preserve and or create long-term stability in national healthcare systems for:

- a. displaced individuals affected by trauma and conflict through:
 - i. psychotherapy and complementary therapeutic approaches,
 - ii. implementing trauma informed care systems,
- b. local healthcare workers working in such conflict-affected zones through:
 - i. offering mental health support services for medical staff,
 - ii. providing emergency response training in vulnerable regions,
 - iii. creating further security protocols in conflict zones,
 - iv. supporting the deployment of already trained local healthcare staff in order to ensure continuous care;

15. **Recommend**s the shift to effectively see the ongoing situation, annual global meetings among the Member States to identify sufficient and insufficient improvement of countries by:

- a. including refugees in national health systems like the National Health Insurance Fund,
- b. establishing measures such as safe routes in underdeveloped countries with undergoing conflicts, which aims to provide the needed medical resources by the usage of delivery equipment through and safe zones that would work as refugee camps,
- c. creating periodic meetings in which countries gather together to ensure that each individual has access to basic healthcare needs and discussing the next steps towards improvement,
- d. proposes the implementation of a report made by the UN agencies in order to verify where the invested funds have been used, preventing them from being invested in other purposes by some countries;

16. **Support**s the use of digital health technologies to enhance the provision of medical services to conflict-affected populations, including by:

- a. expanding access to healthcare for populations in remote, besieged, or otherwise inaccessible areas,
- b. assure fast emergency response, to create efficient healthcare and overcome language barriers,
- c. encouraging the allocation of resources, including from relevant United Nations funds and mechanisms, to support the sustainable digitalization of healthcare systems;

17. **Proposes** positioning mobile medical units to border crossings and high transit zones to ensure that refugees and displaced people receive medical care as soon as possible by:
 - a. supplying these units with essential equipment like trauma kits and diagnostic tools,
 - b. providing rapid triage teams that can identify and get to priority treatment high-risk individuals like pregnant women, elderly people and sick children;
18. **Suggests** conducting non-discriminatory and culturally appropriate training for health providers, in order to:
 - a. lower stigma and discrimination displaced people face when seeking help,
 - b. strengthen trust between health providers and migrant patients,
 - c. encourage consistent care seeking from migrants;
19. **Endorses** states to remain actively seized on the matter.